

Patient Education Practice Guidelines for Health Care Professionals

The Patient Education Practice Guidelines for Health Care Professionals was developed by the Health Care Education Association to provide concise direction for frontline health care providers. Patient education is defined as “A process of assisting consumers of health care to learn how to incorporate health related behaviors (knowledge, skill, attitude) into everyday life with the purpose of achieving the goal of optimal health” (Bastable, 2017, p. 542). Over 10,000 articles and resources were reviewed to identify evidence-based practice for patient education.

Four Components of the Patient Education Process

The guidelines are based on the four components of the patient education process: **assessment, planning, implementation and evaluation (APIE)** (Bastable, 2017). Each component is essential for effective patient education. No component can be skipped or receive lesser attention. In this guideline, specific concise instructions are provided on how to address each of the components.

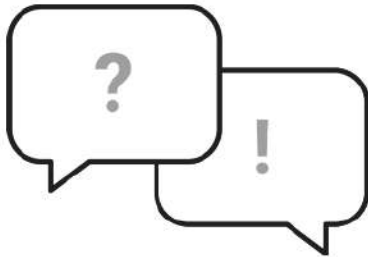
In the APIE process, individualizing education can only be accomplished through **assessment** of the patient (e.g., knowledge, goals, language). The education **plan** focuses on the patient’s priorities in addition to needs identified by the health care professional. **Implementation** uses key learning strategies and can be adapted based on the patient’s response. In the **evaluation** component, the patient’s knowledge/behavior and the health professional’s ability to teach are evaluated.

Overarching Elements

From the literature review, several overarching elements emerged which impact the APIE process. Effective patient education focuses on the concepts of “**patient-centered**” and “**patient engagement**.” Additionally, effective strategies include **plain language** and focusing on **behaviors** and **actions**, not just knowledge.

Best practice **combines all of these elements**. Ideally, education is an **interactive process focusing on the desired patient behavior and patient’s stated priorities** to achieve health goals. Use the guidelines to lead or direct you through the patient education process.

The term “patient education” is defined in this context to be a broad classification that includes not only patients, but also consumers, family, friends, neighbors, guardians, significant other/ partner or anyone else designated to meet care needs.



Frequently Asked Questions

What are the guidelines?

A concise resource for patient education evidence-based practice for frontline health care professionals.

Who should use the guidelines?

Any health care professional who provides patient education.

Where should the guidelines be used?

In any setting where patients learn about how to achieve health care goals.

When should the guidelines be used?

Any time patient instructions are being given such as disease information, test preparation/ results, treatment, accessing care, appointments and resources.

Why should the guidelines be used?

To ensure the implementation of evidence-based patient education thereby assisting health care consumers in achieving optimal levels of health.

How do you use the guidelines?

Follow the steps in each section of the guidelines (assessment, planning, implementation and evaluation) along with explanations, examples, and scripts.



Assessment

Effective patient education is based on a learning needs assessment. Health care professionals assess by interviewing the patient and family, communicating with the medical team and/or observing the patient. An examination of barriers that impact delivery of care is key in the development of a tailored plan to meet the needs, abilities and preferences of the patient. These practices empower patients to change behaviors and are referred to as “patient-centered” care.

Assessment Steps

1. Assess **sociodemographic** information as well as **support system, culture/values/beliefs** and **barriers** to learning.
2. Assess learning needs based on **current health issues, knowledge and worries.**
3. Assess **patient engagement** in learning process (patient's goals and priorities, motivation to learn).
4. Assess **learning preferences** (verbal, written, visuals, multi-media, technology).
5. Consider specific **assessment tools.**

Assessment Steps	Explanations/Examples/Scripts	Ref List
1. Collect Information		
Review culture, social support and sociodemographic information directly with patient or from health record. Includes: • Age • Gender/preferred pronoun • Ethnicity • Social determinants of health: education, socioeconomic status, access to food and health care	• Establish rapport, use caring tone and note body language. • Address culture, values, attitudes, beliefs: » “Do you have any beliefs or feelings related to your health condition that we need to know so we can help you learn to care for yourself?” » “What do you fear most about your sickness?” • Inquire about social support network: » “Who is your family member or friend who can help you with your healthcare?” Note: Higher education and/or social economic status does not indicate better understanding of information.	List 1
2. Current Status		
Address patient concerns and priorities first, then current knowledge and physical abilities.	• “What are you most concerned about?” • “What do you do at home now to care for yourself?” • “Do you know anyone who has this condition?” • “What do you know about caring for someone with diabetes?”	List 2
3. Patient Engagement		
Assess patient confidence related to treatment plan adherence; self-efficacy	• “Based on our discussion, here are the top 3 concerns you have for your health. What would you like to learn first?” • “On a scale of 0 to 5, zero being low and five being high, how would you rate your confidence that you can follow your instructions and care for yourself at home?” • “What would you do if you have questions or doubts?”	List 3